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Characteristics of people who believe in conspiracy theories: Personality disorder traits, paranormal beliefs, and political ideologies

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The present study investigated personality disorder traits, paranormal beliefs, and political ideologies associated with people who believe in conspiracy theories. Through Amazon Mechanical Turk, 133 adults (M = 45.16 years, range = 23–65 years, 84 women, 46 men, three not stated) completed two conspiracy measures: A newly constructed Conspiracy Ideational Scale and the Generic Conspiracist Beliefs Scale. They also completed the Coolidge Axis II Inventory, Revised Paranormal Belief Scale, and Political Beliefs Scale. It was found that personality disorder traits weakly predicted conspiracy theory beliefs, while paranormal beliefs and political beliefs were found to be stronger predictors. The strongest relationship among the measures was between the Conspiracy Ideational Scale and Conservative/Republican-leaning attitudes (r = .60, p < .001). Interestingly, even the Generic Conspiracist Beliefs Scale, designed to be independent of political beliefs, was still strongly associated with people who have Conservative/Republican-leaning attitudes (r = .42, p < .001). It appears that paranormal beliefs and Conservative/Republican leaning-attitudes are better predictors of conspiracy beliefs than personality disorder traits.

Keywords: conspiracy theory believers, personality disorder traits, paranormal beliefs, political ideologies

The purpose of this study was to examine personality traits and attitudes that might predict an individual's belief in conspiracy theories. A conspiracy theory, although not scientifically based, is the general sentiment of a secretive agreement amongst a group of people, often with the intent of accomplishing a malicious goal (e.g., van Prooijen & van Vugt, 2018). Conspiracy theories appear to be more prevalent and popular than ever before. For example, a Google Scholar search of the past two decades on 'conspiracy' yielded about 402,000 citations, while in the previous two decades, there were about 128,000 citations.

Wood and Douglas (2019) proffered that the illusion of the conspiracy theory must be sinister in nature and cast an immense amount of doubt on what the majority of the public believes to be true. Fenster (1999) believed that the essence of a belief in a conspiracy theory was the desire to alert and persuade the masses that there is truth out there that needs to be uncovered. It was the focus of the present study to determine the relationships among personality characteristics, paranormal beliefs, and political ideologies of people who endorse conspiracy theories.

Interestingly, conspiracy theories are not a new phenomenon. A massive fire in Rome in 64 CE was blamed on Emperor Nero, and Nero tried to extinguish these rumors by starting a conspiracy of his own by blaming Christians (Pagán, 2020). Conspiracy theories were also popular in the Americas in the 17th century, as the Puritans believed that they were being attacked by disciples of the devil. Further, it was their belief that when Christianity began to spread in Europe, the devil had moved into the New World (Butter, 2020). Butter observed that the history of conspiracy theories has progressed from orthodox to heterodox knowledge. By orthodox, Butter meant that the conspiracy was originally seen as legitimate, and by heterodox, he meant that conspiracy theories had become stigmatized and were no longer a majority belief. Butter also noted that conspiracy theories have nevertheless been influential over the past two centuries, and they have shaped perceptions of many critical events in history. Butter used the Kennedy assassination and the moon landing as well-known examples of the influence conspiracy theories have had in recent history. Butter emphasized that conspiracy theories continue to be heterodox knowledge, particularly because uncensored social media often makes it difficult for people to differentiate what is true and what is not in public discourse.

Van Prooijen and van Vugt (2018) hypothesized that conspiracy theories could be evolutionary by-products of more basic psychological mechanisms. They proposed that a by-product does not have adaptive features itself, but it can be 'carried along' with certain adaptive mechanisms that do possess adaptive features, like the association between creativity and schizophrenia. Van Prooijen and van Vugt offered two evolutionary hypotheses: The first is that psychological mechanisms such as pattern recognition, threat management, and agency detection have evolutionary value because they protect an organism from danger. They proposed that these mechanisms evolved because the

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consequences of failing to recognize a genuine threat are far greater than the consequences of detecting a threat where none exists. Their second hypothesis was that dangerous coalitions were a genuine threat in the ancestral environment. Again, the failure to detect a dangerous coalition (a false positive) would have been deadly and contrary to evolutionary reproductive success. However, holding the belief that a dangerous coalition existed in the absence of a genuine, dangerous coalition would have had far fewer deadly consequences.

A brief review of the characteristics of conspiracy theory believers

At least three characteristic categories have emerged in association with beliefs in conspiracy theories: (1) Personality disorders, and trait domains and facets (from the *Diagnostic and Statistical Manual of Mental Disorders* [*DSM-5-TR*; American Psychiatric Association, 2022]); (2) Paranormal beliefs; and (3) Political ideology.

Personality disorders, trait domains, and facets

Swami, Weis, Lay et al. (2016) investigated the relationship between the five personality disorder trait domains and their 25 facets with conspiracy beliefs in 259 adult participants. The domains and facets were measured by the Personality Inventory for *DSM-5* (PID-5; Krueger, Derringer, Markon et al., 2012). They found significant and positive correlations between every domain and conspiracy beliefs: Psychoticism (r = .49), Disinhibition (r = .42), Antagonism (r = .41), Negative Affectivity (r = .40), and Detachment (r = .34). They also found significant and positive correlations between all of the 25 facets and conspiracy beliefs (median r = .38; range Risk Taking [r = .24] to Unusual Beliefs and Experiences [r = .48]). It is interesting to note that the latter facet had the strongest correlation with conspiracy beliefs (yet less than the criterion for a strong correlation [r > .50]). This finding may be deemed specious at best because, by definition, conspiracy beliefs are almost always unusual beliefs. The latter phenomenon is inherently redundant in its nature and reveals little about the essential characteristics of those who believe in conspiracy theories. Further, Swami et al. (2016) found no significant differences in conspiracy beliefs as a function of the participants' sex, ethnicity, or education.

Bowes, Costello, Ma, and Lilienfeld (2020) examined four adult samples in a total of 1,927, investigating the relationship between the PID-5, other measures of personality, and three measures of conspiracy beliefs. They found significant and positive correlations between the PID-5 domains and facets, other personality traits, and conspiracy beliefs. However, none of the correlations had a strong effect size (r < .50). The authors concluded, "… the relatively modest or weak effect sizes we have reported, the picture we offer here is best regarded as a fuzzy sketch, ideally one to be fleshed out in future research" (p. 13).

Teličák and Halama (2021) also investigated the relationship between the PID-5 domains, spirituality, religiosity, and conspiracy beliefs in 829 adults. Consistent with Bowes et al.'s (2020) findings, there were only weak relationships among the variables, including spirituality and religiosity. They also concluded that conspiracy beliefs "… are a complex multifactor phenomenon and a consequence of the interaction between personality, cognitive, social, and other factors" (p. 185).

Furnham and Grover (2022) explored the relationship between a measure of 14 *DSM* personality disorders (SCATI; Coolidge, Segal, Cahill, & Simenson, 2010) and conspiracy beliefs in 475 British adults. They found that the Schizotypal, Paranoid, and Borderline PD scales best predicted conspiracy beliefs; however, only the Schizotypal scale reached a medium effect size. They concluded that conspiracy beliefs are associated with a wide range of personality disorders, albeit weakly or at best modestly.

In summary, the relationship between the *DSM-5-TR* personality disorders, trait domains, and facets, and beliefs in conspiracy theories has been found to be complex, multifactorial, and unrevealing at best. As noted previously, studies have found the facet of Unusual Beliefs and Experiences, which is part of the Psychoticism domain, to be the most highly correlated with conspiracy beliefs, which is simply supplying a redundant synonym.

Paranormal beliefs

A paranormal belief can be described as a scientifically impossible phenomenon, such as precognition, psychokinesis, witchcraft, and aliens, etc. (Darwin, Neave, & Holmes, 2011). According to Darwin et al., belief in the paranormal has also been described as a multidimensional construct that merges the ideas of magical thinking, superstition, and religious beliefs. Lobato, Mendoza, Sims, and Chin (2014) examined paranormal and conspiracy beliefs in 455 college students. Their paranormal belief scale consisted of 14 items measuring various beliefs in ESP (e.g., telekinesis, fortune telling, séances, etc.), geomancy (e.g., Bermuda Triangle, crystal power, etc.), astromancy (e.g., astrology, solar and lunar influences, etc.), and fantastic beasts (e.g., Bigfoot, aliens, chupacabra, etc.). They found a strong positive correlation between paranormal beliefs and conspiracy beliefs (r = .52) and concluded that these related beliefs were manifestations of "similar intuitive cognitions" (p. 624). Further, they noted that although there were content differences between these beliefs at a superficial level, they may have an underlying cognitive foundation. Swami et al. (2016) later argued that paranormal beliefs were "akin" to conspiracy beliefs, and this set of anomalous beliefs allowed people to more easily believe in conspiracy theories.

Political ideology

Van Prooijen, Krouwel, and Pollet (2015), in a study with 268 adult participants, found that political extremism (e.g., extreme conservatism and extreme liberalism) and conspiracy beliefs were significantly correlated. Confirming their original hypothesis, those who held extreme political beliefs were more likely to believe in conspiracy theories than those with more moderate political beliefs. The authors speculated that extreme ways of thinking, seeing things as 'black-and-white,' contributed to the susceptibility to conspiracy theories. They also believed that political extremists may use this way of thinking as a coping mechanism when receiving information that creates fear and feelings of uncertainty to

counter feelings of predictability and comfort. They suggested that the relationship between political extremism and conspiracy beliefs may be derived from a greater need to manage uncertainty (possibly stronger for right-wing political beliefs; Jost, Glaser, Kruglanski, & Sulloway, 2003). Galliford and Furnham (2017), in a study with 323 adults, found that those belonging to right-wing political parties were more likely to endorse political conspiracy theories than other political parties. Like Jost et al. (2003), Douglas, Uscinski, Sutton et al. (2019) speculated that politically conservative people endorse conspiracy theories more than liberals because of a personality predisposition, which requires a greater need to manage uncertainty.

The Big Five and conspiracy theory believers

The Big Five personality theory model (Costa & McCrae, 1985) has dominated personality research for four decades, with over eight million current citations, according to Google Scholar. Brotherton, French, and Pickering (2013) found that their measure of general conspiracy theories (i.e., not politically aligned) was weakly and negatively correlated with Conscientiousness (r = – .16) and weakly and positively correlated with Agreeableness (r = .11), but Openness, Extraversion/Introversion, and Neuroticism failed to meet the criterion for a weak correlation (r > .10). In a series of studies, Swami, Chamorro-Premuzic, and Furnham (2010; Swami, Coles, Stieger et al., 2011; Swami, Pietschnig, Tran et al., 2013; Swami et al., 2016) also found weak relationships between conspiracy ideation and the Big Five. In summary, research with the Big Five traits has shown that their relationships to conspiracy beliefs tend to be weak and not consistently replicated.

It does appear that beliefs in conspiracies are a multifactor phenomenon, yet we suspect that this particular belief system represents a persistent and enduring pattern of behavior relating to the world of experience that deviates from most cultural norms. As van Prooijen et al. (2015) suggested, it may be attributed to a "highly structured thinking style" (p. 576), and as Goertzel (1994) hypothesized, conspiracy beliefs may reflect a monological belief system, i.e., a variety of attitudes that are all likely to emanate from a single hierarchical factor. In this regard, consistent and long-lasting beliefs in conspiracies may be akin to a heretofore unrecognized personality disorder (Coolidge & Srivastava, 2026). In part, our present study aimed to measure this belief system using three of the most highly cited characteristics of those who believe in conspiracy theories: personality disorder traits, paranormal beliefs, and political ideology. We believe that doing so might provide a preliminary explanation for those people who not only believe in conspiracies over time but also more readily adopt new conspiracy beliefs

The main theme of the present study was to develop a conspiracy beliefs scale and to determine how it was related to (1) personality disorder traits, (2) paranormal beliefs, and (3) political beliefs, each of which has shown some predictive value in the conspiracy beliefs literature. We also attempted to determine the preliminary concurrent validity of our new scale within an established measure of conspiracy beliefs thought to be independent of political orientation or beliefs.

METHOD

The present study was approved by the University of Colorado, Colorado Springs Institutional Review Board.

Participants and procedure

We recruited English-speaking adults, 18 years and older, who reside in the USA, through Amazon Mechanical Turk (MTurk) by way of Qualtrics. Through informed consent, participants were advised that they would remain anonymous and receive $1.00 if they completed the study. A total of 242 participants completed the study, although 109 (45%) of them either failed the three attention checks or completed the measures in an unreasonably short period of time. Thus, the final sample consisted of 133 participants: 84 females, 46 males, and three others who preferred not to disclose information regarding their sex. The average age of the participants was 45.16 years (range = 23 – 65 years, SD = 12.68 years). The levels of education of the participants were: Doctoral degree (N = 1; 0.8%), Master's degree (N = 23; 17.3%), Bachelor's degree (N = 48; 36.1%), Associate's degree (N = 17; 12.8%), some college but no degree (N = 27; 20.3%), high school graduate/GED (N = 16; 12.0%), and less than a high school degree (N = 1; 0.8%)). Participants' ethnicity included White (N = 95; 71.2%), Black or African American (N = 13; 9.8%), Asian (N = 10; 7.6%), Hispanic (N = 7; 5.3%), and other (N = 8; 6.1%).

Measures

Coolidge Axis II Inventory (CATI)

The CATI is a *DSM-5-TR*-aligned 260-item self-report measure of psychopathology and neurocognitive dysfunction (Coolidge, 2022). The CATI measures in the present study included 14 personality disorder scales (with the number of items on each scale): Antisocial (15), Avoidant (9), Borderline (17), Depressive (8), Dependent (14), Histrionic (14), Narcissistic (17), Obsessive-Compulsive (19), Paranoid (14), Passive-Aggressive (15), Sadistic (9), Schizoid (8), Schizotypal (17), and Self-Defeating (13), as well as a Generalized Anxiety Disorder (15) scale. Each CATI item is answered on a 4-point Likert-type scale ranging from 1 (*Strongly False*) to 4 (*Strongly True*). The CATI has been shown to be reliable and valid in a variety of clinical and non-clinical settings (Coolidge, 2022; Coolidge & Merwin, 1992; Coolidge, Burns, & Mooney, 1995). The CATI personality disorder scales' items cover all of the *DSM-5-TR* chief criteria and associated features for each disorder. A sum was obtained for each of the individual items for each disorder; thus, each CATI personality disorder scale was measured dimensionally by a sum of the total items and not categorically (i.e., met the minimum criteria according to *DSM-5-TR* for the presence of a particular personality disorder).

Conspiracy Ideational Scale (CIS)

The CIS was created by Coolidge and Chappelle (2022), and it was used for the first time in the present study, primarily because conspiracy theories have a tendency to change rapidly. The CIS is a 20-item, true-false single scale that purports to measure beliefs in current conspiracy theories.

Table 1. Conspiracy Ideation Scale

Item	Content
1.	The 2020 USA presidential election was stolen, and Joe Biden is not our legally elected president.
2.	The Washington, DC, protest of January 6 was actually instigated by the Deep State within the U.S. government in order to attack and punish right-wingers, conservatives, and followers of Donald Trump.
3.	Barack Obama's birth certificate was a forgery, and his actual birthplace was Kenya.
4.	The Sandy Hook shooting was a manufactured event to promote gun control.
5.	Bill and Hillary Clinton have assassinated 50 or more of their associates (aka The Clinton Body Count conspiracy).
6.	The Food and Drug Administration is suppressing natural cures to many illnesses, such as cancer and COVID-19 (aka Alternative Therapy Suppression conspiracy).
7.	The HIV/AIDS virus was created to kill gay people.
8.	Fluoridation (fluoride in water supplies) is a plot to slowly poison Americans.
9.	Global warming is a myth and was invented for political reasons.
10.	The Earth is actually flat. There is evidence that has been suppressed to hide that Earth is a single, infinite plane. GPSs are rigged to make pilots believe they are flying around the globe.
11.	COVID-19 vaccines contain a magnet or GPS tracking device that connects to the 5G cellular network. Bill Gates may have been responsible for putting this device in vaccines to track American citizens.
12.	Big agricultural businesses have intentionally hidden data from the public that shows that Genetically Modified Organisms (GMOs) are actually dangerous and cause harm.
13.	Area 51, near Roswell, New Mexico, is a highly classified United States Air Force facility where an alien spaceship crashed in 1947. The government is suppressing information about the spaceship and the aliens who were recovered in the crash.
14.	QAnon is a valid belief that there is a group of Satan worshippers, pedophiles, and some Democrats. Donald Trump has been fighting to protect people from these Satan worshippers, pedophiles, and others.
15.	There is a volcano under Yellowstone in Wyoming. Its eruption will wipe out the United States. The government knows when this will happen, but is suppressing the information.
16.	The Denver International Airport may be the headquarters for the Illuminati, a group of secret individuals who are trying to control the world. It may also have been built by Neo-Nazis, who are concealing underground tunnels and buildings.
17.	Ivermectin, Hydroxychloroquine, and Oleandrin can treat and cure the COVID-19 virus. Big pharmaceutical companies are hiding this fact because they can only make money from vaccines.
18.	The COVID-19 virus was created by the Chinese government, and it did not come from animals.
19.	The destruction of the Twin Towers in New York City on September 11, 2001, was planned in advance by American citizens who may have been working for the US Government. Explosives may have already been inside the World Trade Center before the two plane crashes.
20.	The US moon landing in 1969 was faked, and it was actually filmed on a stage in the USA.

Each item possesses face validity with popular conspiracy beliefs. The maximum CIS score is 20, and the minimum score is 0, with higher scores indicating higher levels of belief in conspiracy theories. See Table 1 for the 20-item CIS.

Generic Conspiracist Beliefs Scale (GCBS)

This scale purports to measure beliefs in conspiracy theories independent of political orientation or beliefs (see Drinkwater, Dagnall, Denovan, & Neave, 2020, for additional details). The GCBS is a 15-item single scale, with each item answered on a 5-point Likert-type scale ranging from 1 (*Definitely not true*), 2 (*Probably not true*), 3 (*Not sure/Cannot decide*), 4 (*Probably true*), and 5 (*Definitely true*). An example of a statement on this scale is, "The government is involved in the murder of innocent citizens and/or well-known public figures and keeps this a secret".

Revised Paranormal Belief Scale (RPBS)

The RPBS (Tobacyk, 2004) was originally created to measure various categories of paranormal beliefs. It is a 26-item scale, with each item answered on a Likert-type scale ranging from 1 (*Strongly disagree*) to 5 (*Strongly agree*). Seven subscales of the RPBS were evaluated (Traditional Religious Belief [4], Psi [4], Witchcraft [4], Superstition [3], Spiritualism [4], Extraordinary Life Forms [3], and Precognition [4]). Tobacyk reported an overall test-retest reliability (4-week interval) of .92. Lange, Irwin, and Houran (2000) reported a median internal reliability of .84 for the seven subscales, ranging from .67 for the Extraor-

Table 2. Internal reliabilities for CATI, Generalized Anxiety Disorder scale, CIS, GCPBS, RPBS with its seven subscales, and PBS

No.	Scale	Cronbach's α value
1.	Antisocial PD	.74
2.	Avoidant PD	.88
3.	Borderline PD	.89
4.	Depressive PD	.88
5.	Dependent PD	.85
6.	Histrionic PD	.81
7.	Narcissistic PD	.87
8.	Obsessive-Compulsive PD	.85
9.	Paranoid PD	.88
10.	Passive-Aggressive PD	.87
11.	Sadistic PD	.65
12.	Schizoid PD	.77
13.	Schizotypal PD	.66
14.	Self-Defeating PD	.84
15.	Generalized Anxiety Disorder	.91
16.	CIS	.91
17.	GCBS	.96
18.	RPBS	.94
19.	Traditional Religious Belief	.91
20.	Psi	.90
21.	Witchcraft	.91
22.	Superstition	.85
23.	Spiritualism	.91
24.	Extraordinary Life Forms	.64
25.	Precognition	.85
26.	PBS	.75

dinary Life Forms subscale to .85 for the Spiritualism subscale. Lange et al. strongly recommended against the interpretation of a single sum of the seven subscales.

Political Belief Scale (PBS)

The PBS is a 31-item scale that purports to measure Conservative/Republican-leaning versus Liberal/Democrat-leaning attitudes (Pyszczynski, Kesebir, Motyl et al., 2018). This scale was modified from the Pyszczynski et al.'s Political Attitudes Questionnaire II, which had an excellent internal reliability of α = .94. Items were modified to become politically polarizing and current. Each item was answered on a True/False scale, where a True answer reflected a Conservative/Republican-leaning statement is "Kneeling during the National Anthem is disrespectful." A Liberal/Democrat-leaning statement is "Vaccines should be mandated to protect the general public." The overall scale score ranges from 31 to 124, with higher scores representing Conservative/Republican-leaning beliefs.

Planned statistical analyses

The statistical analyses will include Cronbach's measure for internal reliability for all scales, a correlation to determine the concurrent validity between the GCBS and RCIS, correlations and multiple linear regression between the CIS and the CATI 14 PD scales, correlations and multiple linear regression between the CIS and the RPBS, correlations between the CIS and the PBS, and an exploratory independent samples *t*-test on extreme political beliefs on the CIS.

RESULTS

Statistical analysis

Internal reliabilities

A summary of the internal reliabilities (Cronbach's α) for the 14 CATI personality disorder scales, the Generalized Anxiety Disorder scale, the CIS, the GCPBS, the RPBS with its seven subscales, and the PBS appears in Table 2. The median α for the 14 CATI PD scales was .85, with a range of .65 to .89. For the other 11 scales, Generalized Anxiety Disorder scale, CIS, GCPBS, RPBS with its seven subscales, and PBS, the mean α was .91, with a range of .64 to .96. 20 of the 26 total scales had α's > .80, and all of the scales had acceptable α levels at > .64.

Concurrent validity

The use of the GCBS, which was created theoretically to be independent of political orientation or beliefs (Drinkwater et al., 2020), allowed us to determine the preliminary concurrent validity of our newly created CIS. There was a strong positive and significant correlation between these two measures (r = .76, p < .001). Interestingly, there was also a moderate positive and significant correlation between the GCBS and the PBS (r = .42, p < .001), which may demonstrate a Conservative/Republican-leading bias even towards general conspiracies. Coupled with the previous correlation between the CIS and the PBS (r = .60), this may indicate that conspiracy beliefs cannot be measured completely independent from political beliefs (or at the very least, conspiracy beliefs cannot be separated from Conservative/Republican-leaning attitudes).

Personality disorders and CIS

We found that 13 of the 14 CATI personality disorder scales were positively correlated with the CIS; however, 12 of these 13 scales only achieved a weak effect size (i.e., r > .10 and r < .27), and only four scales were significant (Borderline, Paranoid, Schizotypal, and Schizoid; Table 3). The median correlation across all 14 CATI PD scales with the CIS was r = .13. The CATI Generalized Anxiety Disorder scale also produced a weak and nonsignificant correlation with the CIS (r = .11).

The 14 CATI personality disorder scales also served as predictor variables in a multiple linear regression with the CIS as the dependent variable. The overall model was significant, $F(14, 118) = 2.52$, p < .01, R = .46, R^2 = .21, adjusted R^2 = .12. Inspection of the standardized beta coefficients revealed only two scales were significant: Avoidant, β = -.44, p < .01 and Paranoid, β = .42, p < .02. These results, coupled with the correlation analyses, are obviously anomalous. First, the two strongest correlations with the CIS were the Schizotypal scale (r = .27, p < .01) and the Paranoid scale (r = .25, p < .05). However, the

Table 3. A summary of correlations between the CIS and the 15 CATI scales

CATI scales	CIS	*p*
Antisocial	.13	> .05
Avoidant	.04	> .65
Borderline	**.20**	< .03
Dependent	.13	> .05
Depressive	.13	> .05
Histrionic	-.04	> .05
Narcissistic	.11	> .05
Obsessive-Compulsive	.15	> .05
Paranoid	**.25**	< .05
Passive-Aggressive	.11	> .05
Sadistic	.13	> .05
Self-Defeating	.16	> .05
Schizotypal	**.27**	< .01
Schizoid	**.22**	< .02
Generalized Anxiety Disorder	.11	> .05

standardized β for the Schizotypal scale was not significant (p = .14). A second anomaly was that the zero-order correlation of the Avoidant scale with the CIS was weak, positive, and not significant (r = .04, p > .05), however in the linear multiple regression its standardized beta coefficient was the strongest with an effect size that was not only medium up to strong but also negative. As Nathans, Oswald, & Nimon (2012) have noted, it is common practice to interpret only the significant beta coefficients, but it may be a highly misleading practice. Coolidge (2021) has suggested always coupling beta coefficients in linear multiple regression in view of the independent variable's individual correlations with the dependent variable. Thus, there was only weak support for the general hypothesis that PDs and generalized anxiety disorder would be good individual predictors of the CIS, although there was more support for this hypothesis for the personality disorder predictors as a group, based on the overall linear regression model with R = .46 and adjusted R^2 = .12.

Paranormal beliefs and CIS

Based on previous literature, we also tested the hypothesis that paranormal beliefs (as measured by the seven subscales of the RPBS) would be positively and significantly correlated with the CIS. All seven subscales were significant at p < .001 and attained a medium to strong effect size. The median correlation was r = .40: Witchcraft, r = .58, Psi, r = .41; Spiritualism, r = .40; Precognition, r = .40; Traditional religious beliefs, r = .38; Extraordinary life forms, r = .35; Superstition, r = .33.

The multiple linear regression model with the seven subscales as the predictors and the CIS as the dependent variable was significant $F(7, 125) = 11.17$, $p < .001$, $R = .62$, R^2 = .39, adjusted R^2 = .35. Upon evaluation of the standardized beta coefficients, it was revealed that only one of the seven subscales was significant: Witchcraft, β = .63, p < .001, as a strong positive predictor. Again, the results of the linear multiple regression's standardized beta coefficients stand in stark contrast to the results of all of the medium to strong individual correlations with the CIS. In contrast to the PD scale findings, it appears that paranormal beliefs are much stronger predictors of beliefs in conspiracy theories than preexisting PD traits.

Political beliefs and CIS

It was also hypothesized that there would be a significant and positive correlation between political beliefs (as measured by the PBS, where high scores indicate Conservative/Republican-leaning attitudes, and low scores indicate Liberal/Democrat-leaning attitudes) and the CIS. The results revealed that there was a strong, positive, and significant correlation (r = .60, p < .001) between the PBS and the CIS. Thus, it appears that conspiracy beliefs are well predicted by one's political leanings. Specifically, Conservative/Republican-leaning attitudes are positively and strongly correlated with conspiracy beliefs.

It is important to note that the GCBS was included in the study because its authors created it to be independent in content from any overt Conservative/Republican-leaning attitudes associated with conspiracy beliefs. A review of the content of the CIS revealed some Conservative/Republican-leaning attitudes. Nevertheless, there was a strong and significant correlation between these two measures (r = .76, p < .001). There was also a moderate correlation between the PBS and the GCBS (r = .42, p < .001), which may demonstrate a Conservative/Republican-leading bias even towards general conspiracies. Coupled with the previous correlation between the CIS and the PBS (r = .60), this may indicate that conspiracy beliefs cannot be measured completely independent from political beliefs (or at the very least, conspiracy beliefs cannot be separated from Conservative/Republican-leaning attitudes).

Exploratory independent samples *t*-test

In an exploratory analysis to test van Prooijen et al.'s (2015) claim that political extremists (either conservative or liberal) endorse conspiracy theories at similar levels, an independent samples t-test was performed between the 20 most extreme scores for Conservative/Republican-leaning attitudes and the 20 most extreme scores for Liberal/Democrat-leaning attitudes on the PBS. It was found that the extreme Conservative/Republican group scored significantly higher (M = 8.80) than the extreme Liberal/Democrat group (M = 0.45) on the CIS, $t(20.28) = -7.59$, p < .001, with a very strong effect size (Cohen's d = 2.46). Thus, we found no evidence for van Prooijen et al.'s claim that political extremists endorse conspiracy theories at similar levels.

DISCUSSION

The current study investigated the extent to which conspiracy theories can be understood from the perspective of personality disorders, paranormal beliefs, and political beliefs. Given the previous and present findings on the overall weak relationship between personality disorders and conspiracy beliefs (e.g., Bowes et al., 2020; Furnham & Grover, 2022; Swami et al., 2016; Teličák & Halama, 2021), it may be preliminarily concluded that personality disorder

traits, as measured by the traditional PD categories (each measured dimensionally as the sum of the items on each scale), are not a productive venue for understanding the minds of people who believe in conspiracy theories. Further, in the Teličák & Halama study, where they used the alternative measure of personality disorders' five domains and their 25 facets, all 30 measures were moderately to strongly predictive of beliefs in conspiracy theories. Thus, how is such a result to be interpreted when all measures are sensitive to conspiracy beliefs but none exhibit specificity? In addition, as noted earlier, previous studies have found the Unusual Beliefs and Experiences facet of the Psychoticism domain to be the most strongly correlated with conspiracy beliefs. This finding is tautological, i.e., unnecessarily repetitive, because conspiracy beliefs by definition are typically unusual beliefs, and thus, this finding does not provide conceptual clarity. As noted earlier, previous research with the Big Five has shown that their relationships to conspiracy beliefs tend not to reach the criterion for a weak relationship ($r < .10$), demonstrate weak relationships ($.30 > r > .10$), these relationships are not consistently replicated, and are generally uninformative, e.g., Swami et al. (2010; 2011; 2013; 2016).

The present study confirmed the excellent internal reliability ($\alpha = .91$) and concurrent validity of the newly created CIS, as the CIS correlated with the well-established GCBS at $r = .76$, $p < .001$. Future CIS studies are certainly warranted. However, it is important to note that while some conspiracy theories remain relatively consistent over time (e.g., the faked US moon landing), new conspiracy theories appear frequently, based upon current events such as disasters, catastrophes, political climate changes, and other human experiences. Thus, the ever-changing nature of conspiracies may present a methodological challenge for their measurement.

Consistent with previous literature, we found that paranormal beliefs were more strongly correlated with conspiracy beliefs than an existing measure of PD traits (i.e., the CATI). With regard to paranormal beliefs, the RPBS witchcraft subscale was found to have the strongest correlation with belief in conspiracy theories ($r = .58$). There were four items in this subscale: "Black magic really exists", "Witches do exist", "Through the use of formulas and incantations, it is possible to cast spells on persons", and "There are actual cases of witchcraft". These items clearly involve something malevolent in nature; therefore, meeting Wood and Douglas' (2019) definition that a conspiracy theory must be sinister in nature, which supports our hypothesis that vulnerability to conspiracy beliefs is a multifactor phenomenon.

One surprising finding was that beliefs in conspiracies were intimately tied to Conservative/Republican attitudes, which was in contrast to the van Prooijen et al. (2015) study, where they found conspiracy beliefs to be associated with political extremism, whether conservative or liberal. They speculated that conspiracy theory believers engaged in this behavior due to fear associated with uncertainty. In the exploratory analysis in the present study for their claim, there was no evidence that extreme conservatives and extreme liberals had similar beliefs in conspiracy theories. It was clear through our analyses that people with Conservative/Republican-leaning attitudes were more likely to endorse conspiracy theories than people with Liberal/Democrat-leaning attitudes.

Interestingly, although not overtly noted by van Prooijen et al. in their study, one of their figures revealed that the strength of conservative attitudes with conspiracy beliefs was approximately 30% higher than that of Liberal attitudes. The latter finding raises the interesting question of why there is a stronger moderating relationship of conspiracy beliefs with Conservative/Republican attitudes than with Liberal ones. Further, we might question why conservative political views are more highly associated with conspiracy beliefs and paranormal beliefs than liberal political views. It does seem relatively clear that the Big Five, the *DSM-5-TR* AMPD, and the *DSM-5-TR* categorical PD model and their symptomatic traits only modestly, at best, capture the nature of those with an increased vulnerability to believing in conspiracy theories. It is also interesting to note that the *DSM-5-TR* categorical PD model begins with a short one-sentence description of each PD. We might add the following: Conspiracy Belief Personality Disorder is a pattern of a current belief in a conspiracy and a vulnerability to new conspiracy beliefs, which disrupts an individual's interpersonal relationships, their education, and/or their occupational functioning.

One major limitation of the present study was the introduction of a new conspiracy beliefs scale, where 11 of the 20 items are directly related to USA issues. Thus, the scale may not be suitable for a more general or non-USA audience. However, nine of our items may be generalizable to other cultures. We believe that the core of our new scale consists of assessing an increased tendency to believe in various conspiracy theories. Future studies may wish to investigate our contention that the scale is dominated by a single component through a principal components analysis. Other limitations included the large number of people ($N = 109$; 45%) excluded from the study who failed the three attention checks or completed the measures in an unreasonably short period of time. Another issue may have been the nature of testing the participants online, because they may have been motivated solely by the remuneration (although only $1.00). The generalizability of the study may also have been limited by having nearly twice as many women ($N = 84$) as men ($N = 46$) and a relatively restricted age range (23 to 65 years old). The sample was also predominately White (71%) from the USA. Future studies may wish to address these limitations to their generalizability in larger and more diverse samples.

In summary, we can arrive at three conclusions: (1) The belief in conspiracy theories is a complex multifactor phenomenon; (2) Popular personality models do not appear to provide a specific understanding of this multifactor phenomenon; and (3) Conspiracy beliefs may reflect a monological belief system, which appears to reflect a consistent and enduring behavioral pattern that may constitute a heretofore unrecognized personality disorder (Coolidge & Srivastava, 2026).

REFERENCES

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). Author.

American Psychiatric Association. (2013). *Diagnostic and statistical manual of mental disorders* (5th ed.). Author.

Bowes, S. M., Costello, T. H., Ma, W., & Lilienfeld, S. O. (2020). Looking under the tinfoil hat: Clarifying the personological and psychopathological correlates of conspiracy beliefs. *Journal of Personality*, *89*(3), 422-436.

Brotherton, R., French, C. C., & Pickering, A. D. (2013). Measuring belief in conspiracy theories: The generic conspiracist beliefs scale. *Frontiers in Psychology*, *4*, 279-279.

Butter, M. (2020). Conspiracy theories in American history. In M. Butter & P. Knight (Eds.), *Routledge handbook of conspiracy theories* (pp. 648-659). Routledge.

Coolidge, F. L. & Srivastava, A. (manuscript submitted; 2026). Does increased vulnerability to conspiracy beliefs constitute a new personality disorder? A preliminary cross-cultural comparison. *Psychology Department, University of Colorado, Colorado Springs, and Department of Humanities and Social Sciences, Indian Institute of Technology Gandhinagar*.

Coolidge, F. L. (2021). *Statistics*: *A gentle introduction.* Sage.

Coolidge, F. L. (2022). *Coolidge Axis II Inventory*: *Manual*. Author.

Coolidge, F. L., Burns, E. M., & Mooney, J. A. (1995). Reliability of observer ratings in the assessment of personality disorders: A preliminary study. *Journal of Clinical Psychology, 51*, 22-28.

Coolidge, F. L., & Chappelle, C. N. (2022). *Conspiracy ideational scale* [Unpublished manuscript]. Department of Psychology, University of Colorado, Colorado Springs.

Coolidge, F. L. & Merwin, M. M. (1992). Reliability and validity of the Coolidge Axis Two Inventory: A new inventory for the assessment of personality disorders. *Journal of Personality Assessment, 59*, 223-238.

Coolidge, F. L., Segal, D. L., Cahill, B. S., & Simenson, J. T. (2010). Psychometric properties of a brief inventory for the screening of personality disorders: The SCATI. *Psychology and Psychotherapy: Theory, Research and Practice, 83*, 395-405.

Costa, P. T., & McCrae, R. R. (1985). *The NEO Personality Inventory Manual*. Psychological Assessment Resources.

Darwin, H., Neave, N., & Holmes, J. (2011). Belief in conspiracy theories: The role of paranormal belief, paranoid ideation, and schizotypy. *Personality and Individual Differences, 50*(8), 1289-1293.

Douglas, K. M., Uscinski, J. E., Sutton, R. M., Cichocka, A., Nefes, T., Ang, C. S., & Deravi, F. (2019). Understanding conspiracy theories. *Political Psychology*, *40*, 3-35.

Drinkwater, K. G., Dagnall, N., Denovan, A., & Neave, N. (2020). Psychometric assessment of the generic conspiracist beliefs scale. *PloS One, 15*(3), Article e0230365.

Fenster, M. (1999). *Conspiracy theories: Secrecy and power in American culture*. University of Minnesota Press.

Furnham, A., & Grover, S. (2022). Do you have to be mad to believe in conspiracy theories? Personality disorders and conspiracy theories. *International Journal of Social Psychiatry*, *68*(7), 1454-1461.

Galliford, N., & Furnham, A. (2017). Individual difference factors and beliefs in medical and political conspiracy theories. *Scandinavian Journal of Psychology, 58*(5), 422-428.

Jost, J. T., Glaser, J., Kruglanski, A. W., & Sulloway, F. J. (2003). Political conservatism as motivated social cognition. *Psychological Bulletin, 129*(3), 339–375.

Krueger, R. F., Derringer, J., Markon, K. E., Watson, D., & Skodol, A. E. (2012). *Personality Inventory for DSM-5 (PID-5)* [Database record]. APA PsycTests. https://doi.org/10.1037/t30042-000

Lange, R., Irwin, H. J., & Houran, J. (2000). Top-down purification of Tobacyk's revised paranormal belief scale. *Personality and Individual Differences*, *29*(1), 131-156.

Lobato, E., Mendoza, J., Sims, V., & Chin, M. (2014). Examining the relationship between conspiracy theories, paranormal beliefs, and pseudoscience acceptance among a university population. *Applied Cognitive Psychology, 28*(5), 617-625.

Nathans, L. L., Oswald, F. L., & Nimon, K. (2012). Interpreting multiple linear regression: a guidebook of variable importance. *Practical Assessment, Research & Evaluation, 17*(9), Article 9.

Pagán, V. E., (2020). Conspiracy theories in the Roman Empire. In M. Butter & P. Knight (Eds.), *Routledge handbook of conspiracy theories* (pp. 531-541). Routledge.

Pyszczynski, T., Kesebir, P., Motyl, M., Yetzer, A., & Anson, J. (2018). *Ideological consistency, political orientation, and variability across moral foundations* [Unpublished manuscript]. Department of Psychology, University of Colorado Colorado Springs.

Swami, V., Chamorro-Premuzic, T., & Furnham, A. (2010). Unanswered questions: a preliminary investigation of personality and individual difference predictors of 9/11 conspiracist beliefs. *Applied Cognitive Psychology, 24*, 749–761.

Swami, V., Coles, R., Stieger, S., Pietschnig, J., Furnham, A., Rehim, S., & Voracek, M. (2011). Conspiracist ideation in Britain and Austria: Evidence of a monological belief system and associations between individual psychological differences and real-world and fictitious conspiracy theories. *British Journal of Psychology, 102*(3), 443-463.

Swami, V., Pietschnig, J., Tran, U. S., Nader, I. W., Stieger, S., & Voracek, M. (2013). Lunar lies: The impact of informational framing and individual differences in shaping conspiracist beliefs about the moon landings. *Applied Cognitive Psychology, 27*(1), 71–80.

Swami, V., Weis, L., Lay, A., Barron, D., & Furnham, A. (2016). Associations between belief in conspiracy theories and the maladaptive personality traits of the Personality Inventory for *DSM-5*. *Psychiatry Research*, *236*, 86-90.

Teličák, P., & Halama, P. (2021). Maladaptive personality traits, religiosity, and spirituality as predictors of epistemically unfounded beliefs. *Studia Psychologica, 63*(2), 175-189.

Tobacyk, J. J. (2004). A revised paranormal belief scale. *International Journal of Transpersonal Studies, 23*(1), 94-98.

Uscinski, J. E. (2018). *Conspiracy theories and the people who believe them*. Oxford University Press.

Van Prooijen, J. W., Krouwel, A. P. M., & Pollet, T. V. (2015). Political extremism predicts belief in conspiracy theories. *Social Psychological & Personality Science*, *6*(5), 570-578.

Van Prooijen, J. W., & van Vugt, M. (2018). Conspiracy theories: Evolved functions and psychological mechanisms. *Perspectives on Psychological Science*, *13*(6), 770-788.

Wood, M. J., & Douglas, K. M. (2019). Conspiracy theory psychology: Individual differences, worldviews, and states of mind. In J. E. Uscinski (Eds.), *Conspiracy theories and the people who believe them* (pp. 245-256). Oxford University Press.

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